

SPECIAL REPORT

American Perspectives on Early Detection of Alzheimer's Disease in the Era of Treatment



In 2017, the *Alzheimer's Disease Facts and Figures Special Report* highlighted the promise of biomarkers, stating, "... we envision a future in which Alzheimer's disease is placed in the same category as other chronic diseases, such as cardiovascular disease or diabetes, which can be readily identified with biomarkers and treated before irrevocable disability occurs."¹⁰⁶² Less than a decade later, we are close to realizing that promise. Highly accurate blood-based biomarker tests for detecting Alzheimer's disease may soon be available in physicians' offices.

There have also been great strides in treatment of Alzheimer's disease. The U.S. Food and Drug Administration (FDA) has approved new treatment options that address the underlying biology and slow the decline of memory, thinking and function in a meaningful way for some people diagnosed with Alzheimer's disease in the early stages. By slowing the progression of Alzheimer's, individuals could have more time to participate in daily life and live independently — underscoring the importance of early detection and diagnosis.

Implications of Early Detection and Diagnosis for Treatment to Slow Alzheimer's Progression

People diagnosed in the earlier stages — mild cognitive impairment (MCI) or mild dementia due to Alzheimer's disease — are the only ones for whom the new anti-amyloid medications donanemab (Kisunla™) and lecanemab (Leqembi®) are approved.¹⁰⁶³ Both medications remove beta-amyloid to reduce cognitive and functional decline, thereby slowing the typical progression of the disease (see Treatments to Slow Alzheimer's Disease section, page 14).

Throughout this Special Report, "anti-amyloid medication" is used interchangeably with "treatments to slow progression." To be eligible for treatment to slow disease progression, an Alzheimer's diagnosis must be confirmed with tests showing elevated levels of beta-amyloid in the brain. Establishing this with diagnostic testing can be a lengthy process, often involving multiple steps and various clinicians.

People considering anti-amyloid treatments should discuss the risks and benefits of all approved therapies and other health care considerations with their physicians to develop a tailored treatment plan.¹⁰⁶³

Considerations include:

- Current anti-amyloid medications are administered as infusions at specialty outpatient clinics.
- Possible side effects of anti-amyloid medications include amyloid-related imaging abnormalities (ARIA), infusion-related reactions, headaches or falls; some of these can be serious.
- Appropriate use recommendations that require magnetic resonance imaging (MRI) scans before select infusions to identify and manage ARIA.
- Incorporation of other approaches, such as medications that manage symptoms or non-drug treatments.
- Insurance coverage of the desired treatment.
- Personal preferences for interventions and care.

Early Detection and Diagnosis Could Ease Other Concerns

In addition to determining eligibility for and facilitating access to treatment, early detection and diagnosis of Alzheimer's disease could have emotional and practical benefits, such as:^{526,1059,1064-1067}

- Reducing anxiety and promoting peace of mind associated with knowing one's status, even in light of a diagnosis, and the improved clarity, education and understanding that follow.
- Empowering individuals through knowledge to promote a sense of self-control and agency over decisions.
- Improving personal and financial planning in conversation with family and caregivers to allow for informed decision-making.
- Promoting healthy behaviors, such as physical activity and a nutritious diet.
- Prompting proactive safety measures, such as managing driving ability, medication adherence and home modifications.
- Creating the potential for better outcomes as a result of earlier intervention and improved quality of life.

Current State of Early Detection and Diagnosis

While not everyone experiencing cognitive decline has or will develop Alzheimer's disease, early detection and diagnosis can offer deeper insight to help navigate potential next steps, including treatment options. Today, early detection and diagnosis of Alzheimer's are closely linked. Unlike screening tests for other diseases that may pick up biological signals before outward physical symptoms appear, detecting and diagnosing Alzheimer's depends on physician assessment, often prompted by the individual's or a family member's observations of memory and thinking problems.

Clinically, early diagnosis is recognized as when an individual meets the requirements for cognitive impairment with early functional impact or mild dementia with functional impairment (Stages 3 and 4; see the Alzheimer's Disease Continuum, page 10, for more information).⁵³ "Functional impact" means self-reported or observed increases in the amount of time it takes to independently complete complex activities of daily living, whereas "functional impairment" describes more noticeable difficulties with everyday tasks.⁵³ Accurate diagnosis currently hinges on combining evidence from medical history, neurological exams, cognitive assessments and brain imaging. No one test can definitively establish that the cause of cognitive symptoms is Alzheimer's or another dementia. This diagnostic complexity currently limits early diagnosis.

Barriers to accessing health care — including logistical issues, e.g., work schedules and access to transportation, staff workforce shortages and financial constraints — present additional challenges to early detection and timely diagnosis. Although primary care physicians (PCPs) can perform standardized cognitive assessments, they may not feel equipped to administer the screening or interpret and discuss the results. Furthermore, PCPs often refer patients to specialists for additional confirmatory testing to evaluate the cause of memory issues before an official diagnosis. Patients may encounter long wait times for comprehensive testing, in part due to the scarcity of dementia specialists. Additionally, brain imaging equipment or specialized expertise to perform other diagnostic tests may not be locally available, further delaying confirmatory diagnosis.

Early Detection With Blood-Based Biomarker Tests Could Lead to More Timely Diagnosis

The limitations of current detection methods, including their complexity and accessibility challenges, highlight the need for more efficient and readily available tools. The latest promising advancement in Alzheimer's detection is blood-based biomarker tests (see the Looking to the Future: Importance of Biomarkers section, page 26). Blood tests are relatively simple to administer in a clinical setting and are commonly used to monitor health status, detect medical conditions and, in some cases, definitively diagnose a disease. Alzheimer's blood-based biomarker tests signal a shift to a more accessible method of early detection, potentially prevailing over currently expensive and/or invasive methods that are not always within reach for patients.

Blood tests for Alzheimer's disease offer potential advantages. Experts believe blood-based biomarker tests could improve the accuracy and speed of diagnosis when used as a complement to other testing, offering a viable path to earlier Alzheimer's detection and diagnosis. As of the writing of this Special Report, such tests are limited to use in

At a Glance: Alzheimer's Disease Biomarkers and Biomarker Testing

What are biomarkers?

Biomarkers are measurable biological changes that can:

- Indicate the presence or absence of disease.
- Assess the risk of developing symptoms of a disease.

What are biomarkers for Alzheimer's disease?

Biomarkers for Alzheimer's disease include the proteins beta-amyloid and phosphorylated tau (p-tau). Researchers are studying other possible biomarkers.

What biomarker tests are used to detect Alzheimer's disease?

When Alzheimer's disease is suspected, a physician can order medical tests to measure biomarkers, specifically brain imaging scans or samples of cerebrospinal fluid (CSF).

What do Alzheimer's biomarker tests look for?

Current biomarker tests primarily measure accumulation of beta-amyloid or p-tau as evidence of the hallmark brain changes of Alzheimer's disease.

What's the state of biomarker testing?

Current research is investigating novel, simple medical tests to detect biomarkers of Alzheimer's disease. Blood-based biomarker tests that detect p-tau or beta-amyloid are the furthest along and the closest to being widely available. A number of researchers are also exploring retinal biomarkers (e.g., proteins associated with retinal neurodegeneration and changes in the thickness and cell structure of the retina), cerebrovascular changes or even changes in sleep quality. Other emerging biomarker research includes examining components in saliva and the skin for signals that may indicate early biological changes in the brain.

Can biomarker tests diagnose Alzheimer's disease?

No, the presence of these biomarkers alone is not sufficient to determine an Alzheimer's diagnosis at this time.

specialty care clinics to aid in the diagnostic workup of symptomatic individuals and are still being validated in clinical trials. The Alzheimer's Association® does not recommend their use in asymptomatic individuals at this time.

Much remains to be learned about Americans' willingness to pursue testing, diagnosis and treatment to slow Alzheimer's disease progression, which was the impetus for the 2025 Alzheimer's Disease Facts and Figures Special Report survey.

Attitudes About Early Detection and Treatment of Alzheimer's Disease

This year's Special Report offers new insights into the public's knowledge, interest and views regarding detection and treatment of Alzheimer's disease.

To better understand these topics, the Alzheimer's Association commissioned Versta Research to survey U.S. adults age 45 and older. The survey aimed to identify key knowledge gaps, measure public awareness, and uncover attitudes and beliefs about Alzheimer's across different demographic groups.

Key Findings

Attitudes about early detection and diagnosis

The vast majority of Americans believe diagnosis at the early stages of Alzheimer's disease is important.

- Nearly all (99%) Americans said it is important to diagnose Alzheimer's in the early stages of the disease.
- Most (59%) also viewed cognitive screening for Alzheimer's or other dementia as a very important aspect of preventive health care.

Most Americans would want to know if they have Alzheimer's disease when they have no symptoms or minor symptoms of cognitive decline.

- Nearly 4 in 5 Americans (79%) would want to know if they had Alzheimer's disease before experiencing symptoms or before symptoms interfere with daily activities.

Consistent with interest in early diagnosis, nearly all Americans would want to undergo a simple test, e.g., a blood-based biomarker test, to detect Alzheimer's disease if it were available.

- More than 9 in 10 Americans would definitely or probably want a simple medical test if it were available.
 - 91% would want testing before symptoms appear (presymptomatic).
 - 95% would want testing when experiencing early symptoms (postsymptomatic).
- Four in 5 Americans (80%) said they would ask for a simple medical test rather than wait for their doctor to suggest it; this was consistent across all populations surveyed.

The top reason to seek a simple medical test is the potential for earlier health care intervention.

- More than 4 in 5 Americans (83%) would want to undergo simple medical testing for Alzheimer's because it would allow for earlier treatment and care.
- Other leading reasons included that detection allows time for planning (76%), encourages action to preserve existing cognitive function (68%) and helps with understanding what is happening (67%).

Implications for future insurance coverage are the major concern related to testing that detects Alzheimer's.

- More than 2 in 5 Americans (44%) had concerns about insurance companies not covering subsequent care after testing.
- Other leading concerns about being given a simple medical test included test accuracy (41%), losing confidence in one's ability to carry out daily activities (40%) and the cost of testing (39%).

Attitudes about anti-amyloid medications

The option for treatment to slow progression elevates the importance of an early Alzheimer's diagnosis.

- Nearly 3 in 4 survey respondents (73%) said being able to take medication to slow the progression of Alzheimer's during its early stages would influence their feelings about an early diagnosis.

If diagnosed with Alzheimer's, most Americans would want medication to slow the progression of the disease and highly value information about it.

- More than 9 in 10 Americans (92%) would probably or definitely want a medication that could slow the progression of Alzheimer's disease following a diagnosis.
 - Nearly 2 in 3 Americans (64%) knew that anti-amyloid medications targeting underlying causes to slow disease progression exist.
- Information about treatments that slow the progression of the disease (72%) and information about Alzheimer's disease (69%) top the list of most valued information people want to receive following a diagnosis.
- Survey participants expressed equally strong interest in other options to manage and treat Alzheimer's disease — 94% would want medications that lessen symptoms, and 90% would want education and support for lifestyle changes.

Treatment risks and logistics do not diminish interest in anti-amyloid medication.

- Nearly 3 in 5 Americans (58%) would accept moderate or high levels of medication risk to slow the progression of Alzheimer's disease.
- Three in 4 Americans (74%) said visiting an outpatient clinic at least once a month for treatment would not affect their interest in an anti-amyloid medication.

In 2022, the Association published *Appropriate Use Recommendations for Blood Biomarkers in Alzheimer's Disease*, which provides guidance for clinicians and researchers using these tests in clinical trials, as well as priorities for future research in this area.²⁷⁷

Establishing Clinical Practice Guidelines to Bridge Scientific Advances and Clinical Care

The Alzheimer's Association is building an updated library of clinical guidance that distills the latest scientific evidence and translates it into clear and actionable recommendations for clinical practices. This process involves close collaboration with clinical and subject-matter experts, methodologists, peer organizations, early-career researchers and patient representatives. Expert panels will move from evidence to recommendations using a transparent methodology called the Grading of Recommendations Assessment, Development and Evaluation (GRADE) approach to vet the quality of evidence used to inform recommendations. Numerous reputable organizations, including the World Health Organization, the American College of Physicians and the Cochrane Collection follow the GRADE principles and format for systematic reviews and clinical guidelines.¹⁰⁷¹

Recognizing that specialists need more detailed and comprehensive recommendations to provide high-quality care and promote early detection and diagnosis, the Association is currently preparing guidelines on:

- Blood-based biomarker tests: Specialized health care settings (anticipated in 2025).
- Cognitive assessment tools: Primary health care settings (anticipated in 2025).
- Clinical implementation of Alzheimer's disease staging criteria and treatment (anticipated in 2026).

These guidelines are expected to be updated annually to reflect the rapidly evolving science, new evidence and practical experiences of health care professionals.

Fostering Better Conversations About Testing, Diagnosis and Treatment

Clinical practice guidelines provide the framework for high-quality dementia care, but the foundation of trust is built on communication among patients, caregivers and clinicians.

Effective communication is absolutely necessary for improving testing, diagnosis and treatment for Alzheimer's and other dementias. Primary Care Physicians (PCPs) are often on the frontlines of dementia diagnosis and care. However, the 2020 *Alzheimer's Disease Facts and Figures* Special Report found that PCPs feel inadequately prepared to care for patients with Alzheimer's and other dementias, citing difficulty answering questions, making diagnoses and staying current with the latest developments in

management, treatment and screening/testing.¹⁰⁷² Beyond these challenges faced by PCPs, communication barriers exist between patients and clinicians. Both groups are often uncomfortable raising concerns or discussing cognitive symptoms, which can hinder crucial next steps for detection, diagnosis and treatment.

Enhancing patient-clinician dialogue is paramount not only for improving communication about next steps but for engaging patients and caregivers in the dementia care journey. To achieve this, it is essential to develop and invest in training that increases comfort with difficult conversations and prioritizes listening to patient concerns, responding with empathy and providing clear, jargon-free explanations of complex medical information.¹⁰⁷³ Furthermore, training should teach techniques for collaborative decision-making that integrate patient preferences and values into the discussion.⁹¹⁸

Future clinical guidelines will offer recommendations on how to broach the topic of blood-based biomarkers with patients. In the meantime, there is a growing body of language to help clinicians, including PCPs and dementia specialists, convey information about diagnosis and treatment in a balanced and realistic way. In 2024, The Alzheimer's Association Clinical Meaningfulness Workgroup presented recommendations and suggested language to help health care providers communicate clearly, accurately and empathetically about newly approved anti-amyloid medications to patients diagnosed with early Alzheimer's disease and their caregivers.⁵⁸

The workgroup's recommendations for discussing and personalizing care plans encapsulated the following core themes:

- Balanced, accurate communication.
- Eligibility and personalization.
- Informed consent and risks.
- APOE genetic testing.
- Ongoing monitoring and safety.
- Financial and logistical considerations.
- Support for caregivers.
- A holistic care approach.

Many of these themes are directly applicable to eventual discussions about early detection. For example, patients and caregivers will need accurate information about how tests work, the meaning of test results, and the risks and benefits associated with receiving positive test results, which may lead to early diagnosis.

The Alzheimer's Association provides a range of resources to support health systems and clinicians in these critical areas, including materials on early detection and diagnosis, management of Alzheimer's and other dementias, care planning and support services. Prioritizing communication

efforts and using available resources creates a dementia care environment where patients and caregivers feel heard, respected and empowered — ultimately resulting in higher quality of care and strengthening the patient-clinician relationship. For a complete listing of available Association resources to support health systems and clinicians, visit alz.org/professionals/health-systems-clinicians.

Examining Potential Ethical Considerations Associated with Advances in Biomarker Testing

The evolving ability to detect protein signals of Alzheimer's disease in presymptomatic individuals, particularly through blood-based biomarker tests, raises important ethical considerations.^{281,1066,1074} As stated earlier in the Special Report, no guidance today recommends biomarker testing for individuals who are not experiencing cognitive symptoms. Experts must establish a clear delineation between detection, which measures indicators of potential risk, and formal diagnosis of Alzheimer's disease, which is a multifaceted process involving cognitive screening, other assessments and, ultimately, clinical judgment.

Possible ethical considerations are heightened by the anticipated FDA approval of these tests for use in primary care settings, where health care professionals may not be as familiar with the critical distinction between detection and diagnosis as specialists are, nor as equipped to accurately convey this to patients as they weigh the benefits and risks of such tests. The health care community can learn from approaches used in clinical research settings, where presymptomatic biomarker information is carefully disclosed to study participants and care partners.^{281,1074} A pattern has been observed of participants in research studies sharing biomarker test results with their care partners, a practice that may well continue when test results are available in the clinic.^{1066,1074}

Pre-counseling patients is an additional proactive approach to respond to possible concerns related to biomarker test results. This approach can set realistic expectations about what these tests can and cannot do, explain the possible implications of results both now and in the future, and educate patients on the distinctions between detection and diagnosis of Alzheimer's disease. Laying this groundwork for patients is vital given the increasing availability of medical test results, including blood tests and imaging, through patient portals — often without sufficient context or clinical interpretation. Premature release of results to patients before physicians are able to provide context and interpretation can lead to misinterpretation and emotional distress.

Furthermore, the potential for discrimination based on the results of biomarker testing, whether in medical settings, the workplace or the community, must be carefully considered to determine how and with whom to share

results. In the clinical research setting, concerns about stigma and discrimination are cited as reasons not to share biomarker test results.^{1066,1074}

This year's Special Report found that individuals are pragmatic about the use of simple medical tests, such as blood-based biomarker tests, for Alzheimer's detection. They stated that they would use the results to plan ahead, be more aware of potential symptoms and seek treatment earlier if symptoms appear.

Advancing Legislative and Policy Initiatives to Secure Insurance Coverage for Current and Future Biomarker Tests

Currently, insurance coverage for biomarker testing, which includes imaging and blood tests, has not kept pace with scientific discoveries and progress in treatment. The majority of states do not require insurance carriers to cover biomarker testing, which gives private insurance plans significant latitude in what they cover. As a result, individuals in these states may not have access to necessary biomarker testing. State Medicaid programs may elect to not cover biomarker testing for a wide variety of reasons, further challenging access. Existing health care disparities and challenges to obtaining a dementia diagnosis may be exacerbated if new biomarker testing opportunities cannot be accessed.

“By ensuring access to necessary biomarker testing, states can reduce the time it takes to receive a diagnosis and enable access to new disease-modifying treatments and care planning.”

With new evidence in the field of biomarker testing and FDA consideration of blood-based amyloid biomarker testing tools expected in the near future, the Association is engaging state governments to ensure adequate coverage of biomarker tests and future access to diagnosis and treatment — reflecting the Association's commitment to these priorities.

The Alzheimer's Association and the Alzheimer's Impact Movement (AIM) are part of a national coalition of patient advocates committed to ensuring insurance coverage for comprehensive biomarker testing. National coalition members include the American Cancer Society Cancer Action Network (ACS CAN), the ALS Association and the Arthritis Foundation. AIM is working with policymakers to advance and pass state legislation requiring insurers to cover biomarker testing (Figure 27). As of January 2025:

- 16 states require coverage in both public and private insurance plans (AZ, CA, GA, IA, IL, IN, LA, KY, MD, MN, NM, NY, RI, OK, PA, TX).
- Two states require private pay only (AR, CO).
- Two states require public pay only (CT, FL).

Appendices

End Notes

- A1. Racial and ethnic identifiers: Facts and Figures keeps the racial and ethnic terms used in source documents when describing study findings. When not referring to data from specific studies, adjectives such as “Black,” “Hispanic” and “White” may be used (for example, Black populations and Hispanic communities).
- A2. Estimated prevalence (number and proportion) of Americans age 65 and older with Alzheimer’s dementia for 2025: The estimated 7.2 million individuals ages 65 years and older with Alzheimer’s dementia and the estimated numbers of individuals with Alzheimer’s in each age group were reported from a study that used data from the Chicago Health and Aging Project (CHAP) in combination with population projections from the U.S. Census.²⁹³ The number, 7.2 million, is higher than estimated from previous study that also combined CHAP and U.S. Census data. This is because the more recent study used updated Census projections and incorporated information from Hispanic/Latino American individuals. The proportion of the population with Alzheimer’s dementia (among people age 65 and older and by age group) is calculated using as the numerators the numbers of people with Alzheimer’s dementia, as reported by the recent study in CHAP.²⁹³ The denominators were the U.S. Census population projections for the specific age groups of interest.
- A3. Differences between CHAP and HRS-HCAP estimates for Alzheimer’s dementia prevalence: The number of people estimated to have any form of dementia in the U.S. in 2016 from the Health and Retirement Study’s (HRS) Harmonized Cognitive Assessment Protocol (HCAP; 4.92 million) is lower than the CHAP estimate of how many people were living with Alzheimer’s dementia only (6.07 million).¹⁷³ This is because of differences in dementia ascertainment between the two studies: both studies used scores on batteries of cognitive tests, but the HRS-HCAP study additionally required an informant report of functional impairment (i.e. disability). Because the more stringent threshold for dementia in HRS-HCAP may miss people with mild Alzheimer’s dementia, the Association believes that the larger CHAP estimates may be a more relevant estimate of the burden of Alzheimer’s dementia in the United States.
- A4. Criteria for identifying people with Alzheimer’s or other dementias in the Framingham Heart Study: From 1975 to 2009, 7,901 people from the Framingham Study who had survived free of dementia to at least age 45, and 5,937 who had survived free of dementia until at least age 65 were followed for incidence of dementia.³⁴⁰ Diagnosis of dementia was made according to the Diagnostic and Statistical Manual of Mental Disorders, 4th Edition (DSM-IV) criteria and required that the participant survive for at least 6 months after onset of symptoms. Standard diagnostic criteria (the NINCDS-ADRDA criteria from 1984) were used to diagnose Alzheimer’s dementia. The definition of Alzheimer’s and other dementias used in the Framingham Study was very strict; if a definition that included milder disease and disease of less than six months’ duration were used, lifetime risks of Alzheimer’s and other dementias would be higher than those estimated by this study.
- A5. Projected number of people with Alzheimer’s dementia, 2020-2060: This figure comes from the CHAP study.²⁹³ Other projections are somewhat lower (see, for example, Brookmeyer et al.¹⁰⁷⁹) because they relied on more conservative methods for counting people who currently have Alzheimer’s dementia.⁴³ Nonetheless, these estimates are statistically consistent with each other, and all projections suggest substantial growth in the number of people with Alzheimer’s dementia over the coming decades.
- A6. Annual mortality rate due to Alzheimer’s disease by state: Unadjusted death rates are presented rather than age-adjusted death rates in order to provide a clearer depiction of the burden of mortality for each state. States such as Florida with larger populations of older people will have a larger burden of mortality due to Alzheimer’s — a burden that appears smaller relative to other states when the rates are adjusted for age.
- A7. Number of family and other unpaid caregivers of people with Alzheimer’s or other dementias: To calculate this number, the Alzheimer’s Association started with data from the Behavioral Risk Factor Surveillance System (BRFSS) survey. Since 2016, all states and the District of Columbia utilized the BRFSS caregiver module. This module identified respondents age 18 and over who had provided any regular care or assistance during the past month to a family member or friend who had a health problem, long-term illness or disability. The module asks a series of follow-up questions, including asking the caregiver to identify what the main health problem, long-term illness, or disability that the person they care for has. One of the reported condition categories is “Alzheimer’s disease, dementia, or other cognitive impairment.” In the BRFSS surveys conducted in 2019 and after, an additional follow-up question was included, asking if the caregiving recipient also had dementia in addition to their main condition. Prior to 2019, the survey did not include caregivers of recipients for whom dementia was not their main condition, so these numbers were imputed using data collected in 2019 by the National Alliance for Caregiving (NAC)/AARP survey. The NAC/AARP survey asked respondents age 18 and over whether they were providing unpaid care for a relative or friend age 18 or older or had provided such care during the past 12 months. Respondents who answered affirmatively were then asked about the health problems of the person for whom they provided care: 11% of respondents reported dementia as the main condition of their care recipient, while 26% of all respondents reported the presence of dementia. Using this ratio in combination with BRFSS data, the Alzheimer’s Association was able to determine the percentage of adults in all states and the District of Columbia who are caregivers for individuals living with Alzheimer’s or another dementia. These percentages were applied to the estimated number of people age 18 and older in each state in July 2024, using U.S. Census Bureau data available at: <https://www.census.gov/programs-surveys/popest/data/tables.html>. This resulted in a total of 11,926 million Alzheimer’s and dementia caregivers across all 50 states and the District of Columbia.
- A8. Number of hours of unpaid care: The BRFSS survey asks caregivers to identify, within five time frames, the number of hours they provide care in an average week. Using the method developed by Rabarison and colleagues,⁵¹⁷ the Alzheimer’s Association assumed the midpoint of each time frame was the average number of hours for each caregiver within that time frame and then calculated the overall average number of hours of weekly care provided by dementia caregivers in each state. This number was then converted to a yearly average and multiplied by the number of caregivers in each state⁴⁷ to determine the total number of hours of care provided. When added together, across all 50 states and the District of Columbia, the total number of hours provided by Alzheimer’s and dementia caregivers is 19.161 billion hours.
- A9. Value of unpaid caregiving: For each state, the hourly value of care was determined as the average of the state minimum hourly wage¹⁰⁷⁶ and the most recently available state median hourly cost of a home health aide. (For Nevada, the minimum wage used was the average of the minimum wage for those who are not provided health insurance and the minimum wage for those who are provided health insurance).⁹⁹² The average for each state was then multiplied by the total number of hours of unpaid care in that state⁴⁸ to derive the total value of unpaid care. Adding the totals from all states and the District of Columbia resulted in an economic value of \$413.452 billion for dementia caregiving in the United States in 2024.